08 — Ethics, Privacy, and QI Determination

> This document is guidance for planning. It is **not legal advice**. Confirm every item with the faculty advisor, the institution's IRB, and the clinic's compliance or privacy officer.

1. QI vs. research determination

Academic QI projects usually need a formal **determination** from the student's institution, either "QI, not human-subjects research" or IRB review. Get it **before** collecting any data, including baseline chart audits.

Features of this project that point to QI: - The aim is to improve local practice using an established standard (AHC-HRSN). - Everyone eligible gets the improved process. There is no randomization or withholding. - Data are used for local improvement.

Features that may call for IRB review: - An intent to publish generalizable findings. Many IRBs still allow QI publication, so ask. - Collecting data beyond routine care, such as patient interviews or surveys for research. - The vulnerable population. People experiencing homelessness may be treated as needing added protections.

Action: submit the institution's QI determination form with this charter, protocol, and measurement plan.

2. Privacy (HIPAA)

- The clinic is likely a HIPAA covered entity. The student needs a role in the clinic's workforce (for example, a student or volunteer agreement) or a data use arrangement to access PHI.
- **Minimum necessary:** the QI logs use a **study ID** only, with no names, DOB, addresses, or phone numbers. The study-ID-to-patient key stays at the clinic, locked, and separate from the logs.
- Dates: keep `visit_date` for weekly charts inside the clinic. For any data leaving the clinic (thesis, publication), convert dates to `week` numbers and remove exact dates.
- Phone numbers from Q14 stay on the paper form and in the clinic record. **Never put them in the CSV logs or this repository.**
- **Never commit real patient data to GitHub.** This repo is for templates and synthetic data only. `.gitignore` blocks `data/real/`.
- Small cells: with about 30 patients, suppress or merge cells with n < 5 in reports to reduce re-identification risk.

3. Consent and autonomy

- Screening is part of routine care, so a separate research consent is normally not required for QI. The patient may still decline any or all items without any effect on care.
- Ask about desire for help separately from risk. Patients may decline referrals.
- Do not give written materials on safety resources without asking whether it is safe for the patient to carry them.

4. Safety disclosures and mandated reporting (Virginia)

- **Adults 60+ or incapacitated adults:** Virginia law makes certain professionals mandated reporters of suspected abuse, neglect, or exploitation to Adult Protective Services (Va. Code § 63.2-1606). Confirm which clinic roles this applies to.
- **Other adults (IPV):** confirm with compliance whether any injury-reporting rules apply to the clinicians. Otherwise, follow a patient-centered response: hotline, safety planning, and patient choice.
- **Minors** are out of scope (adults only). If a minor is encountered, follow child protective services reporting rules.
- Document the clinic's final policy in 03 §6 before go-live.

5. Equity considerations

- Screen everyone the same way. That consistency is the purpose of the project.
- Provide language access (interpreters and translated forms).
- Avoid documentation that could be used against patients. Keep QI logs minimal and coded.
- Include a patient or lived-experience advisor in reviewing wording and acceptability.

6. Checklist

- [ ] Institutional QI/IRB determination letter on file
- [ ] Clinic data-access agreement / workforce status for student
- [ ] Compliance sign-off on safety pathway and reporting
- [ ] Data storage location approved (encrypted, clinic-controlled)
- [ ] Data retention/destruction plan for the linking key